DERMALUXE

BY MEDICARE · SKIN & HAIR CLINICS

ఏలూరు · Eluru

STUDY MATERIAL · TRAINER MANUAL

Skin Care Treatments

స్కిన్ కేర్ ట్రీట్‌మెంట్స్ — 30 రోజుల స్టడీ మెటీరియల్

30 training days · Sundays holiday · daily theory, hands-on, safety, key points, homework and a professional tip. Trainer: Dr. Meghana Valeti, MD DVL.

DAY PLAN · రోజువారీ ప్రణాళిక

01	Welcome, clinic protocol & professional grooming
02	Skin anatomy & physiology
03	Skin types, Fitzpatrick scale & skin analysis
04	Consultation, consent, photography & records
05	Cleansing, exfoliation & the basic facial protocol
06	Product knowledge & home care counselling
07	Chemical peel science — how peels really work
08	Glycolic & lactic peels — hands-on
09	Salicylic & mandelic peels — oily and acne skin
10	TCA, combination peels & frosting endpoints
11	Acne: types, grading & comedone extraction
12	Acne marks & scars — protocols
13	Pigmentation science — melasma, PIH, tanning
14	Brightening protocols & sun protection counselling
15	Laser physics & laser safety
16	Diode laser hair removal — settings & technique
17	LHR practical — face, body & special areas
18	PICO / Q-switched — pigment & tattoo assist
19	Carbon laser facial (Hollywood peel)
20	Hydrafacial & medifacials
21	MNRF & fractional CO2 assist — texture & scars
22	Anti-ageing basics — Botox, fillers, threads, HIFU
23	Mesotherapy, glutathione & skin boosters — assist
24	Complications, emergencies & how to handle them
25	Treatment planning, packages & case studies
26	Client communication, objections & retention
27	Clinic operations, WhatsApp flow & aftercare system
28	Revision + full live-client day
29	Mock practical, viva prep & portfolio
30	Final practical exam, viva & career guidance

Day 01

1 of 30 · Sundays holiday

Welcome, clinic protocol & professional grooming

స్వాగతం, క్లినిక్ ప్రోటోకాల్ & ప్రొఫెషనల్ గ్రూమింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Understand how a doctor-led aesthetic clinic runs, and your role in it
- Learn the DermaLuxe hygiene, sterilisation and safety protocol
- Present yourself like a clinic professional — uniform, hands, voice, body language

THEORY · థియరీ

How an aesthetic clinic works

A patient's journey is: enquiry (WhatsApp/walk-in) → consultation with the dermatologist → treatment plan → procedure → aftercare → follow-up. A therapist touches almost every step: you prepare the room and machine, position the client, assist or perform the procedure under supervision, explain aftercare and make the follow-up call. Your calm, clean, confident handling is what makes a client trust the clinic.

Hygiene & sterilisation

Three levels: cleaning (soap + water removes visible dirt), disinfection (70% isopropyl alcohol or hospital-grade disinfectant on surfaces, trolleys, machine heads) and sterilisation (autoclave for metal instruments — comedone extractors, forceps). Gloves are single-use, changed between clients and after touching anything non-clinical. Hand hygiene before gloves and after removing them. Fresh linen and headband for every client. Needles and blades go straight to the sharps container — never re-cap.

Professional grooming

You are selling skin and hair results — you must look the part. Clean pressed uniform, closed shoes, hair tied back, short filed nails without chipped polish, mild or no perfume, fresh breath, no heavy jewellery on hands. Greet every client standing, with their name. Phone stays out of the treatment room.

Hands-on today

- Tour of the clinic: reception, consultation room, treatment rooms, machine area, sterilisation zone
- Practise hand hygiene: 6-step handwash, glove on/off without contamination
- Set up a treatment trolley from scratch and have it checked
- Practise greeting and seating a client (role play, 3 rounds)

Safety & contraindications

- Never reuse a needle, blade or single-use tip
- Disinfect machine handpieces between clients
- Report any spill, breakage or skin injury to the senior immediately

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 70% IPA for surfaces
- Autoclave for metal instruments
- Gloves: one client = one pair
- Uniform + tied hair + short nails, every day

Homework

Write down the full patient journey of your clinic in 6 steps, and list 5 things in your own grooming you will change from tomorrow.

Grooming & professional tip

First impression = 7 seconds. Clean hands and a warm 'Namaste, చెప్పండి' win more trust than any certificate on the wall.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 02

2 of 30 · Sundays holiday

Skin anatomy & physiology

చర్మ నిర్మాణం & పనితీరు

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Name the layers of skin and what each layer does
- Understand barrier function, pH and why products sting or burn
- Connect anatomy to treatment depth — why a peel or laser works where it does

THEORY · థియరీ

The three layers

Epidermis (top, ~0.1 mm) — the barrier, renews itself every 28 days; contains melanocytes that make pigment. Dermis (middle) — collagen, elastin, blood vessels, nerves, sweat and oil glands; this is where anti-ageing and scar treatments work. Hypodermis (fat) — cushioning and contour. Treatments are classified by how deep they reach: superficial (epidermis) = peels, Hydrafacial; medium (upper dermis) = MNRF, fractional laser; deep = surgical, HIFU (SMAS).

Barrier, pH and TEWL

The stratum corneum is bricks (dead cells) and mortar (lipids). Healthy skin pH is 4.5–5.5 — slightly acidic. Harsh soap, over-exfoliation or too many actives break the mortar, water escapes (trans-epidermal water loss) and skin becomes sensitive, red and reactive. A damaged barrier must be repaired BEFORE any peel or laser — otherwise you get burns and pigmentation.

Melanin & the sun

Melanocytes sit in the basal layer and pass melanin to skin cells as an umbrella against UV. Indian skin (Fitzpatrick IV–V) has very reactive melanocytes: any inflammation — a pimple, a scratch, an aggressive peel — can trigger post-inflammatory hyperpigmentation (PIH). This single fact decides most of our settings and protocols.

Hands-on today

- Label a skin cross-section diagram from memory (2 attempts)
- Feel and compare barrier quality on 3 volunteers: oily, dry, sensitised
- Map treatment depth on the diagram: peel, Hydrafacial, MNRF, CO2, HIFU

Safety & contraindications

- Never treat a skin with a broken barrier (stinging, redness, flaking) — repair first
- Explain PIH risk to every Indian-skin client before any procedure

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Epidermis renews ~28 days
- Skin pH 4.5–5.5
- Fitzpatrick IV–V = high PIH risk
- Repair barrier before any active treatment

Homework

Draw the skin cross-section with the 3 layers and mark where a superficial peel, MNRF and HIFU act.

Grooming & professional tip

Clients don't want anatomy — they want to be understood. Learn to say it simply: 'Mee skin barrier weak ga undi, mundu repair chesi tarvata treatment chestam.'

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 03

3 of 30 · Sundays holiday

Skin types, Fitzpatrick scale & skin analysis

స్కిన్ టైప్స్, ఫిట్జ్‌ప్యాట్రిక్ & స్కిన్ అనాలిసిస్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Classify any client's skin type and Fitzpatrick phototype correctly
- Run a full skin analysis — visual, touch, dermoscope and AI
- Record findings in a way the doctor can use

THEORY · థియరీ

Skin types

Oily (shiny T-zone and cheeks, large pores, acne-prone), dry (tight, flaky, fine lines), combination (oily T-zone, normal/dry cheeks), sensitive (reacts, burns, redness), normal. Type is not permanent — season, products, hormones and treatments change it. Always classify today's skin, not last month's.

Fitzpatrick phototype

I–II very fair (burns, rarely tans), III light brown (sometimes burns), IV moderate brown — most South Indians, V dark brown, VI deeply pigmented. This decides laser energy, peel strength and PIH precautions. Higher type = lower energy, more passes, more cooling, longer gaps.

Analysis workflow

Clean the face first (make-up distorts everything). Look under a good light at the T-zone, cheeks, jawline. Assess: oil, hydration, texture, pores, pigmentation, active acne, marks, scars, sensitivity, sun damage. Use a dermoscope for pigment pattern and a magnifier for comedones. AI skin analysis on the website gives scores you can show the client — it is a conversation tool, not a diagnosis.

Hands-on today

- Analyse 3 volunteers: write skin type, Fitzpatrick, 3 findings and 2 recommendations each
- Take standard photos: front, left 45°, right 45° with fixed light and distance
- Run the AI analysis and compare with your own reading

Safety & contraindications

- Never diagnose a disease — describe what you see and refer to the dermatologist
- Always ask about isotretinoin, recent peels/lasers, pregnancy and allergies

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Most Eluru clients: Fitzpatrick IV–V
- Cleanse before analysing
- Fixed light + distance for photos
- AI scores = talking tool, not diagnosis

Homework

Analyse 2 family members and write a full analysis card for each.

Grooming & professional tip

Photos are your proof. A client who sees Day 0 vs Day 30 side by side books the next package without being asked.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 04

4 of 30 · Sundays holiday

Consultation, consent, photography & records

కన్సల్టేషన్, కన్సెంట్, ఫోటోలు & రికార్డులు

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Run a structured consultation that gets the real concern out
- Take legally sound consent and honest before/after photos
- Maintain client records the clinic can rely on

THEORY · థియరీ

The consultation frame

Open (name, what brings you today), explore (how long, what have you tried, what worries you most), history (medical, medication, allergies, previous procedures, pregnancy/breastfeeding), expectation (what result do you want, by when), explain (what is realistic, how many sessions, downtime), close (plan + next step). Never promise 100% clearance and never quote a price the doctor has not approved.

Consent

Consent is informed: procedure name, expected benefit, number of sessions, possible side effects (redness, swelling, pigmentation, rarely burns/scarring), alternatives and aftercare, in a language the client reads. Signed and dated before the first session, with a fresh consent for any new procedure. Photo consent is separate — never post a client photo without a written, ticked permission.

Records

One file per client: analysis card, consent, treatment log (date, area, machine, settings, reaction), products sold, photos, follow-up notes. Settings written down are what let you increase safely at the next session. 'I remember' is not a record.

Hands-on today

- Role play 3 consultations: acne teenager, melasma housewife, pre-wedding bride
- Fill a consent form completely and get it checked for gaps
- Photograph a volunteer at 3 angles and file them with the record

Safety & contraindications

- No consent = no procedure. No exceptions.
- Pregnancy, isotretinoin in last 6 months, active infection, keloid history → stop and ask the doctor

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Consent before every NEW procedure
- Photo consent is separate
- Log every setting you use
- Never promise 100%

Homework

Write a consultation script in Telugu and English for an acne client, 10 lines each.

Grooming & professional tip

Listen twice as much as you talk. The client's real worry (a wedding, a photo, a comment someone made) is what makes them come back.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 05
5 of 30 · Sundays holiday

Cleansing, exfoliation & the basic facial protocol
క్లెన్సింగ్, ఎక్స్‌ఫోలియేషన్ & బేసిక్ ఫేషియల్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Perform a complete, professional clinical facial start to finish
- Choose the right cleanser and exfoliation for each skin type
- Master hand movements, pressure and timing

THEORY · థియరీ

Step order and why

Double cleanse (oil/micellar then gel) → analyse → exfoliate (mechanical or enzymatic) → steam/softening if needed → extraction if indicated → treatment (mask/serum/device) → soothing → moisturiser → sunscreen. Order matters: actives on unclean skin do nothing, and extraction on unsoftened skin causes bruising and PIH.

Choosing exfoliation

Mechanical scrubs: avoid on active acne and sensitive skin. Enzyme (papain, bromelain): gentle, good for sensitive. AHA (glycolic, lactic): dry, dull, pigmented. BHA (salicylic): oily, comedonal, acne. Frequency: oily 1–2 weekly, dry/sensitive every 10–15 days.

Hands and pressure

Work in upward, outward strokes, both hands symmetrical, constant light pressure, never dragging the eye area. Keep one hand on the client at all times — it feels professional and safe. Room: clean linen, dim light, no phone, comfortable temperature, client's hair fully covered.

Hands-on today

- Full clinical facial on a partner, timed 45 minutes, with checklist
- Practise the 8 classic massage movements on the face and neck
- Set up and reset the room in under 5 minutes

Safety & contraindications

- No steam on rosacea, active acne flare or very sensitive skin
- Never extract without softening; stop after 2 failed attempts on one comedone

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Double cleanse always
- Salicylic for oily, lactic for dry
- Facial ~45 min
- Sunscreen is part of the treatment

Homework

Write your own 12-step facial protocol card and practise twice at home.

Grooming & professional tip

Your touch is the brand. Slow, confident, symmetrical hands make a ₹2,000 facial feel like ₹5,000.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 06

6 of 30 · Sundays holiday

Product knowledge & home care counselling

ప్రొడక్ట్ నాలెడ్జ్ & హోమ్ కేర్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Know the main actives, what they do and who they suit
- Build a simple AM/PM home routine for any client
- Counsel without prescribing medicines

THEORY · థియరీ

The core actives

Vitamin C (morning, brightening, antioxidant), niacinamide (oil control, barrier, pigment, very well tolerated), hyaluronic acid (hydration), ceramides (barrier repair), AHA/BHA (exfoliation), retinoids (night, turnover, acne, ageing — start low, go slow), azelaic acid (acne + pigment, pregnancy friendly), sunscreen (the single most important product).

Sunscreen counselling

SPF 30–50 PA+++, applied 2 finger-lengths for face and neck, 15 minutes before going out, reapplied every 3–4 hours outdoors. Indoors near windows still needs it. Gel/matte for oily, cream for dry, tinted for pigmentation (visible light protection). Without sunscreen, every pigmentation treatment fails — say this in every consultation.

Building a routine

AM: cleanser → (vitamin C or niacinamide) → moisturiser → sunscreen. PM: cleanser → treatment (retinoid or exfoliant, alternate nights) → moisturiser. Change ONE thing at a time, every 2 weeks. Never stack retinoid + strong acid on the same night for a beginner.

Hands-on today

- Build AM/PM routines for 3 profiles: oily acne teen, dry mature, melasma
- Demonstrate the correct sunscreen quantity on a volunteer
- Explain 3 products in simple Telugu to a partner

Safety & contraindications

- Never name or suggest prescription medicines (retinoid strengths, antibiotics, steroids) — that is the doctor's job
- Patch test any new active on clients with sensitive skin

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Sunscreen = non-negotiable
- Niacinamide suits almost everyone
- Retinoid at night, start twice a week
- One change every 2 weeks

Homework

Write a one-page home-care card in Telugu for a pigmentation client.

Grooming & professional tip

Sell routines, not bottles. A client who follows a simple 4-step routine gets results — and results sell the next package.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Chemical peel science — how peels really work
కెమికల్ పీల్ సైన్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Explain peel depth, pH and free acid value in your own words
- Match peel type to concern and Fitzpatrick type
- Set up a complete, safe peel station

THEORY · థియరీ

What a peel does

A controlled chemical injury removes dead and damaged cells and triggers repair — faster turnover, more collagen, more even pigment. Depth decides result and risk: very superficial (stratum corneum) = glow, minimal downtime; superficial (full epidermis) = pigmentation, acne; medium (papillary dermis) = scars, deeper lines — doctor territory in Indian skin.

pH, concentration and free acid

A 30% glycolic peel at pH 3.5 is much gentler than 30% at pH 1.8. Free acid value tells you how much acid is actually active. Never judge a peel by percentage alone. Time on skin is your second dial — start with the shortest recommended time on a first session and build up.

Indian-skin rules

Fitzpatrick IV–V: prefer superficial peels, prime the skin 2–4 weeks before with a gentle routine, avoid sun for 5 days after, use strict sunscreen, space sessions 3–4 weeks apart. Aggressive peels on dark skin cause PIH that takes months to clear and destroys the client's trust.

Hands-on today

- Identify 6 peel bottles: acid, %, pH, indication, neutraliser required or not
- Set up a peel station: peel, neutraliser, fan, cold compress, timer, gauze, gloves
- Do a patch test behind the ear on a volunteer and read it at 24 h

Safety & contraindications

- Always have the neutraliser open and ready before the first drop of peel
- Never peel over active infection, open wounds, or recent waxing/threading (48 h)

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Depth = result + risk
- pH and free acid > percentage
- Start with shortest time
- 3–4 weeks between sessions

Homework

Make a peel comparison chart: glycolic, lactic, mandelic, salicylic, TCA — depth, indication, downtime.

Grooming & professional tip

Confidence in peels comes from preparation, not bravado. A prepared station is what lets you stay calm if skin frosts early.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 08

8 of 30 · Sundays holiday

Glycolic & lactic peels — hands-on

గ్లైకోలిక్ & లాక్టిక్ పీల్స్ — ప్రాక్టికల్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Perform a glycolic peel end to end under supervision
- Recognise correct endpoints and when to stop early
- Handle the post-peel 7 days confidently

THEORY · థియరీ

Glycolic acid

Smallest AHA molecule, penetrates fast — dull skin, mild pigmentation, fine lines, open pores. Typical clinic strengths 20–35% for beginners on Indian skin, 3–5 minutes, always neutralised (sodium bicarbonate solution or water depending on the brand). Expect mild stinging at 30–60 seconds and light erythema.

Lactic acid

Larger molecule, slower and gentler, also hydrating — ideal for dry, sensitive or first-time clients and for the face + neck combination. 30–50%, 3–7 minutes. A very good 'trust builder' first peel before moving to stronger agents.

Endpoints and aftercare

Stop at: even light pink, intense burning the client cannot tolerate, any white frosting (too deep for these peels), or blotchy erythema. Neutralise immediately, cold compress, soothing serum, moisturiser, sunscreen. Aftercare: no scrubs/actives/waxing for 5–7 days, no sun, no hot water, no picking flakes, sunscreen every 3–4 hours.

Hands-on today

- Observe the trainer do a full glycolic peel with commentary
- Perform one glycolic peel on a live client under direct supervision
- Write the post-peel instruction card and explain it to the client in Telugu

Safety & contraindications

- Protect eyes, nostrils and lips with petroleum jelly before application
- Neutraliser within arm's reach; stop the timer, not the conversation

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Glycolic 20–35%, 3–5 min
- Lactic 30–50%, gentler
- Frosting = too deep for AHA
- Sunscreen every 3–4 h for a week

Homework

Write the exact step list you followed today, with timings, from memory.

Grooming & professional tip

Tell the client what they will feel before they feel it. 'Konchem manta vastundi, 30 seconds lo taggutundi' prevents panic and builds trust.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 09
9 of 30 · Sundays holiday

Salicylic & mandelic peels — oily and acne skin
సాలిసిలిక్ & మాండెలిక్ పీల్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Select and perform a salicylic peel for acne-prone skin
- Use mandelic acid safely on sensitive and darker skin
- Combine peel with an acne home-care plan

THEORY · థియరీ

Salicylic acid (BHA)

Oil-soluble, so it goes inside the pore — comedones, oily skin, active acne, back and chest acne. 20–30%, self-neutralising (it dries into a white pseudo-frost of crystals; that is NOT true frosting). 3–5 minutes. Mild anti-inflammatory and antibacterial action, so acne calms within days.

Mandelic acid

Large molecule from bitter almonds — slow, even penetration, very low PIH risk, ideal for Fitzpatrick V, sensitive skin, rosacea-prone and first-timers with pigmentation + acne. Often 20–40%, well tolerated, can be repeated every 2–3 weeks.

Peel series planning

Acne clients need a series: 4–6 sessions every 2–3 weeks, with home care (salicylic cleanser, niacinamide, non-comedogenic moisturiser, gel sunscreen) in between. Warn about a possible purge in weeks 1–2. Photographs at every session keep the client motivated.

Hands-on today

- Perform a salicylic peel on an oily/acne volunteer under supervision
- Perform a mandelic peel and compare client sensation with yesterday's glycolic
- Draw a 6-session plan with dates for an acne client

Safety & contraindications

- Salicylic: avoid in aspirin allergy and in pregnancy
- Never peel over inflamed, pustular, infected acne — treat the infection first (doctor)

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Salicylic = inside the pore
- Salicylic pseudo-frost is normal
- Mandelic = safest for dark skin
- Acne peels: 4–6 sessions

Homework

Prepare an acne client counselling script covering purge, sessions and home care.

Grooming & professional tip

Acne clients are emotional clients. Every session, show them one photo of improvement — it stops them from quitting in week 2.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 10

10 of 30 · Sundays holiday

TCA, combination peels & frosting endpoints

TCA, కాంబినేషన్ పీల్స్ & ఫ్రాస్టింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Understand TCA and combination peels and your role in them
- Read frosting levels correctly
- Assist the doctor safely in a medium-depth procedure

THEORY · థియరీ

TCA basics

Trichloroacetic acid coagulates protein — the skin turns white (frosting). 10–15% is superficial; above that is medium depth and is performed by the dermatologist in Indian skin. Not neutralised — it self-limits, so time and number of coats decide depth. Used for deeper pigmentation, scars and for TCA CROSS on ice-pick scars.

Reading frosting

Level I: speckled light frost with visible pink background (superficial). Level II: uniform white with pink showing through (epidermal/upper dermal). Level III: solid dense white (dermal — risk of scarring and PIH). In dark skin we stop at Level I–II. Learn to see frost appear and call it out loud during the procedure.

Combination peels

Branded blends (e.g. Jessner-type, glycolic + retinoid, salicylic + mandelic) give a controlled, multi-acid action with predictable downtime. Follow the manufacturer protocol exactly — priming, coats, timing, neutralising. Combination peels are a good bridge between a simple AHA and a medium peel.

Hands-on today

- Observe a doctor-performed TCA or combination peel and log every step
- Identify frosting levels from clinical photographs (10 images)
- Prepare and check a medium-peel tray

Safety & contraindications

- Never perform TCA above 15% yourself — assist only
- Have the doctor in the room for any peel that frosts

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- TCA is not neutralised
- Level I–II frost only in dark skin
- Follow brand protocol exactly
- Downtime 5–7 days

Homework

Make a frosting reference sheet with what you would do at each level.

Grooming & professional tip

The skill in peels is stopping early. Clients forgive a slow result; they never forgive a burn.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Acne: types, grading & comedone extraction
యాక్నే రకాలు, గ్రేడింగ్ & కమెడోన్ ఎక్స్‌ట్రాక్షన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Grade acne correctly and know what a therapist may and may not treat
- Perform safe comedone extraction
- Explain acne causes and triggers to a client

THEORY · థియరీ

Why acne happens

Four factors: excess sebum (hormones), blocked follicle (dead cells), C. acnes bacteria and inflammation. Triggers in our clients: humidity and sweat, oily hair products, helmet friction, comedogenic makeup, high-glycaemic and dairy diets, PCOS in women, stress and picking.

Grading

Grade I: comedones only (blackheads/whiteheads). Grade II: papules and few pustules. Grade III: many pustules, nodules. Grade IV: nodulocystic, scarring. Therapists handle Grade I and mild II with facials, peels and extraction. Grade III–IV are medical — doctor consultation first, no aggressive handling.

Extraction technique

Cleanse → soften (steam or warm compress 5–8 min, or enzyme) → sterile comedone extractor or gloved fingers with gauze → gentle even pressure around, never on top → release after 2 failed attempts → antiseptic → soothing mask → sunscreen. Never squeeze inflamed pustules or nodules: that is how scars and PIH are made.

Hands-on today

- Grade 6 clinical acne photographs
- Extraction practice on a live client under supervision (comedones only)
- Write an acne trigger checklist and use it in a role-play consultation

Safety & contraindications

- No extraction on inflamed, pustular or cystic lesions
- Stop if bleeding starts; never dig
- Refer Grade III–IV and any scarring acne to the dermatologist

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 4 causes: oil, block, bacteria, inflammation
- Therapist = Grade I–mild II
- Soften before extracting
- 2 attempts maximum per lesion

Homework

List 10 acne triggers you can spot in a Eluru client's daily routine.

Grooming & professional tip

Teach clients the 'no picking' rule with a reason: 'Prathi sari picking chesthe oka mark ekkuva, 3 nelalu untundi.' Rules with reasons get followed.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Acne marks & scars — protocols
యాక్నే మార్క్స్ & స్కార్స్ ప్రోటోకాల్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Tell the difference between a mark and a scar and set the right expectation
- Plan a combination protocol for marks and for scars
- Track and photograph progress over months

THEORY · థియరీ

Marks vs scars

PIH (brown marks) and PIE (red marks) are pigment and vessels — flat, they fade with time, peels, brightening and strict sun protection. Scars are a change in skin texture: ice-pick (deep narrow), boxcar (sharp edges), rolling (tethered waves), hypertrophic/keloid (raised). Texture never fades on its own — it needs collagen remodelling.

Protocols

Marks: mandelic/salicylic/glycolic series + niacinamide + azelaic + sunscreen; 4–6 sessions; visible change in 6–8 weeks. Scars: MNRF or fractional CO2 (doctor), subcision for rolling, TCA CROSS for ice-pick, and peels only as a support. Realistic result: 50– 70% improvement over 4–6 sessions spread across 6 months — never promise 100%.

Consultation honesty

Show the client the difference on their own face with a mirror and light. Explain months, not weeks. Photograph at every visit under fixed light. Clients who understand the timeline stay; clients who are promised miracles leave after session two.

Hands-on today

- Classify scars on 3 volunteers or photo sets
- Write a 6-month plan for a Grade II acne client with marks and boxcar scars
- Practise the mirror conversation: marks vs scars

Safety & contraindications

- Active acne must be controlled before scar treatment
- Keloid history → no aggressive resurfacing; refer to the doctor

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- PIH/PIE = pigment, fades
- Scars = texture, needs remodelling
- 50–70% over 6 months
- Photograph every session

Homework

Prepare a 'marks vs scars' explanation card in Telugu with two example photos.

Grooming & professional tip

Underpromise and over-deliver. If you say 60% and deliver 70%, you get a referral. Say 100% and you get a complaint.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Pigmentation science — melasma, PIH, tanning

పిగ్మెంటేషన్ సైన్స్ — మెలాస్మా, PIH, టానింగ్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Differentiate the common pigmentation patterns on Indian skin
- Understand why melasma relapses and how to counsel for it
- Build a pigmentation treatment plan with the doctor

THEORY · థియరీ

Common patterns

Melasma: symmetrical brown patches on cheeks, forehead, upper lip; hormone, heat and light driven; chronic and relapsing. PIH: brown marks after inflammation (acne, injury, waxing). Tan: diffuse darkening on exposed areas, reversible. Periorbital melanosis (dark circles): pigment + shadow + vessels + hollowness — treat only the right component. Freckles/lentigines: sun-induced spots.

Why melasma is different

Melanocytes are hyperactive, there is vascularity and often a dermal component. It flares with sun, visible light, heat (cooking, travel), hormones (pregnancy, OCP), and aggressive treatment. Management is control, not cure: gentle peels, tranexamic-based protocols by the doctor, strict tinted sunscreen, heat avoidance and patience.

Building a plan

Prime 2–4 weeks (barrier + sunscreen + brightening) → gentle peel series every 3–4 weeks → maintenance. Add PICO or Q-switched toning only under the dermatologist, with low fluence. Review at 8 weeks with photographs. Aggressive lasering of melasma in Indian skin makes it worse — this is the single biggest mistake in our market.

Hands-on today

- Identify pigmentation type on 5 clinical photos + 2 live clients
- Use the dermoscope to see epidermal vs dermal pigment patterns
- Draft a 3-month melasma plan and present it to the trainer

Safety & contraindications

- Never promise permanent clearance of melasma
- Any sudden, single, changing dark patch → doctor review (rule out other causes)

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Melasma = control, not cure
- Heat and visible light matter, not just UV
- Tinted sunscreen for melasma
- Low and slow in Fitzpatrick IV–V

Homework

Write a melasma counselling script covering relapse, sunscreen and realistic timelines.

Grooming & professional tip

Give melasma clients a 'flare plan' for summer and travel. Clients who feel supported through a flare stay for years.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 14

14 of 30 · Sundays holiday

Brightening protocols & sun protection counselling

బ్రైటనింగ్ ప్రోటోకాల్స్ & సన్ ప్రొటెక్షన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Deliver a complete brightening facial/peel protocol
- Counsel sun protection so the client actually follows it
- Understand glutathione and other brightening add-ons and their limits

THEORY · థియరీ

Brightening actives

Vitamin C, arbutin, kojic acid, azelaic acid, niacinamide, tranexamic acid (topical), liquorice, retinoids. They work on different steps of melanin production, which is why combinations work better than a single hero product — but layering too many causes irritation and, in Indian skin, more pigment.

Clinic protocol

Brightening facial: cleanse → gentle enzyme/AHA exfoliation → brightening mask/serum with light massage → soothing → sunscreen. Brightening peel series: mandelic or lactic, 4–6 sessions, 3 weeks apart, with home care. Expect visible glow immediately and real tone change in 6–8 weeks.

Sun protection that works

Most clients under-apply. Teach the two-finger rule, reapplication every 3–4 hours, and physical measures: hat, scarf, umbrella, car window film, avoiding 11 am – 4 pm sun. For melasma, tinted (iron-oxide) sunscreen for visible light. Track compliance at every visit — ask them to show you their bottle.

Hands-on today

- Perform a full brightening facial on a live client
- Teach sunscreen application to 3 people and check the quantity they apply
- Compare 3 clinic sunscreens: texture, finish, suitability

Safety & contraindications

- Glutathione injections/drips are medical — never recommend or administer on your own
- Stop brightening actives if the skin becomes red, itchy or stings

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Combination > single active
- Two-finger rule
- Reapply every 3–4 h
- Real tone change: 6–8 weeks

Homework

Make a one-page sun-protection card in Telugu that a client can stick on their mirror.

Grooming & professional tip

Ask clients to send you a photo of their sunscreen bottle on WhatsApp. What gets checked gets done.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 15

15 of 30 · Sundays holiday

Laser physics & laser safety

లేజర్ ఫిజిక్స్ & సేఫ్టీ

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Explain wavelength, fluence, pulse width and spot size simply
- Follow laser room safety like a professional
- Prepare, check and shut down a laser machine correctly

THEORY · థియరీ

Four dials

Wavelength (nm) decides what the light is absorbed by and how deep it goes — 755 alexandrite, 808 diode (hair), 1064 Nd:YAG (deeper, safer in dark skin), 1320/1064 picosecond (pigment). Fluence (J/cm^2) is energy per area — the power. Pulse width (ms) is how long the energy is delivered — longer is safer in dark skin. Spot size decides depth and speed.

Selective photothermolysis

Choose a wavelength absorbed by the target (melanin in hair, pigment in a spot) more than by surroundings, and a pulse shorter than the target's cooling time. The target heats and is destroyed; the skin around it stays safe. In dark skin the epidermis also holds melanin — so we reduce fluence, lengthen pulse and cool aggressively.

Laser room safety

Wavelength-specific goggles for client, operator and everyone in the room. Warning sign and closed door, no reflective jewellery, no flammable products (alcohol) on the skin, smoke evacuator for ablative work, machine on standby whenever not firing, foot pedal only under your own foot, test shot on a discreet area, documented settings for every session.

Hands-on today

- Identify the machines in the clinic: wavelength, indications, contraindications
- Set up the laser room to safety standard and be audited
- Do a test shot on a volunteer's discreet area with the trainer and observe reaction at 10 minutes

Safety & contraindications

- Never fire without correct goggles on everyone
- Never treat over tattoos, moles or recently tanned skin
- Know where the emergency stop is on every machine

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 1064 Nd:YAG safest in dark skin
- Longer pulse = safer
- Test shot always
- Goggles for everyone in the room

Homework

Write the safety checklist for your laser room, 12 points, and memorise it.

Grooming & professional tip

The operator who does a test shot every time never has a bad week. Shortcuts in laser are not speed — they are risk.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Diode laser hair removal — settings & technique

డయోడ్ లేజర్ హెయిర్ రిమూవల్ — సెట్టింగ్స్ & టెక్నిక్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Select safe starting parameters by Fitzpatrick type and body area
- Perform correct handpiece technique with cooling
- Counsel the client on the full session course

THEORY · థియరీ

How it works

808 nm diode light is absorbed by melanin in the hair shaft and follicle, heats it and damages the stem cells in the bulge. Only hair in the anagen (growing) phase responds — that is why 6–8 sessions, 4–6 weeks apart on the face and 6–8 weeks on the body, are needed. Grey, white and very fine vellus hair have no melanin and do not respond.

Choosing parameters

In-motion mode: lower fluence, many passes, comfortable and safer for Fitzpatrick IV–V. Static mode: higher fluence, single pass, faster on coarse hair. Typical starting ranges — face 8–12 J/cm^2, underarms/bikini 10–14, arms/legs 12–16, always adjusted after a test shot. Higher Fitzpatrick → lower fluence, longer pulse, more cooling. Record every setting.

Preparation and technique

Shave the area (never wax/thread for 4 weeks), clean, dry, mark the grid. Contact cooling at 0–5 °C, handpiece flat and full contact, overlap 10–15%, systematic grid so nothing is missed or double-treated. Ask about pain every pass. Endpoint: perifollicular erythema and mild oedema — that is success, not damage.

Hands-on today

- Test shot + full underarm treatment on a live client under supervision
- Practise grid technique and overlap on a marked area
- Record a complete treatment log with settings

Safety & contraindications

- No LHR over tattoos, moles, active infection, recent tan, or on isotretinoin (6 months)
- Stop immediately on grey-white change, blistering or severe pain
- Goggles for everyone, every shot

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 808 nm diode, 6–8 sessions
- Shave, never wax, before session
- Endpoint = perifollicular erythema
- Fitz V: low fluence, long pulse, cool hard

Homework

Make a parameter card for 6 body areas at Fitzpatrick IV and V.

Grooming & professional tip

Count your passes out loud. Systematic operators never leave a missed patch — and missed patches are the #1 LHR complaint.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
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Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

LHR practical — face, body & special areas

LHR ప్రాక్టికల్ — ఫేస్, బాడీ & స్పెషల్ ఏరియాలు

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Treat all common areas confidently with correct positioning and draping
- Manage pain, anxiety and modesty professionally
- Handle male clients, beard shaping and body work

THEORY · థియరీ

Area-wise notes

Upper lip and chin: fine hair, hormonal cause common (PCOS) — refer for evaluation; low fluence, protect lips and nostrils. Underarms: mark the pyramid, check for deodorant residue. Bikini: strict draping, chaperone, consent, lower fluence on thin skin. Legs and back: large areas, plan the grid and time. Beard shaping for men: mark with the client sitting, confirm the line twice.

Comfort and dignity

Explain each step before touching. Expose only the area being treated. Use a modesty towel, keep the door locked, and have a female staff member present for intimate areas. Ask permission before repositioning a limb. Offer a break after painful passes.

Session course management

Book the next session before the client leaves. Expect shedding at 7–14 days — tell them it looks like regrowth but is hair falling out. Between sessions: shaving allowed, no waxing/plucking/threading, sunscreen on exposed areas. Photograph hair density every 3 sessions.

Hands-on today

- Two supervised treatments: one facial area, one body area
- Practise draping and positioning for a bikini/underarm case (on a mannequin or with consent)
- Book and diary a 6-session course with dates

Safety & contraindications

- No treatment on sunburnt or freshly tanned skin — wait 2–4 weeks
- Cool the skin immediately if the client reports sharp, lasting pain

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Shedding at 7–14 days is normal
- No waxing/threading during the course
- Chaperone for intimate areas
- Rebook before they leave

Homework

Write the pre- and post-care instruction card for LHR in Telugu and English.

Grooming & professional tip

Modesty and consent are part of the treatment. A client who feels respected refers her sister, her mother and her friends.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 18

18 of 30 · Sundays holiday

PICO / Q-switched — pigment & tattoo assist

PICO / Q-స్విచ్డ్ — పిగ్మెంట్ & టాటూ

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Understand picosecond and Q-switched technology and their uses
- Assist safely in pigment and tattoo sessions
- Deliver correct aftercare and set expectations

THEORY · థియరీ

Technology

Q-switched (nanosecond) and picosecond lasers deliver energy so fast that pigment particles shatter photo-acoustically with minimal heat. Wavelengths: 1064 nm for deeper/darker targets and toning in dark skin, 532 nm for superficial red/brown, 694/755 for greens and blues in tattoos. Fragments are cleared by the body's immune cells over weeks.

Indications and limits

Lentigines, freckles, Hori's naevus, tattoo removal, carbon toning, and cautious melasma toning at very low fluence by the dermatologist. Tattoos need 6–12 sessions 6–8 weeks apart; greens, blues and light colours are stubborn; amateur tattoos clear faster than professional ones. Never promise complete tattoo clearance.

Your role and aftercare

Prepare the room and goggles, clean the area, apply numbing as per protocol and timing, assist during the session, cool after, apply antibiotic/soothing ointment per doctor, dress if needed. Aftercare: no picking, keep dry, sunscreen, no swimming/gym for 48 h, report any blister immediately.

Hands-on today

- Observe and assist a full pigment session start to finish
- Set up a tattoo session tray and safety zone
- Write the aftercare card and explain it to a client

Safety & contraindications

- Immediate whitening is expected in tattoo work; blistering is not — report it
- Never treat a mole; any changing mole goes to the doctor

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 1064 nm safest in dark skin
- Tattoo: 6–12 sessions
- Do not promise 100% clearance
- Strict sunscreen after every session

Homework

Prepare an expectation-setting script for a tattoo removal enquiry.

Grooming & professional tip

Photograph tattoos with a ruler in frame. Objective progress shots stop arguments about 'kanipinchatledu'.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Carbon laser facial (Hollywood peel)
కార్బన్ లేజర్ ఫేషియల్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Perform the carbon laser facial end to end
- Understand who it suits and who it does not
- Position it commercially as an event/glow treatment

THEORY · థియరీ

How it works

A medical carbon lotion is applied and settles into pores; the Q-switched/picosecond laser targets the carbon, which absorbs the energy and is blasted away with oil, debris and dead cells. The controlled heat also tightens pores and stimulates collagen slightly. Zero downtime, instant glow — the classic pre-event treatment.

Protocol

Cleanse → apply carbon evenly, avoid eyes and lips → dry 5–10 minutes → goggles → pass 1 (low fluence, carbon removal) → pass 2 (toning) as per protocol → clean residue → soothing serum/mask → moisturiser → sunscreen. 30–40 minutes total. Series of 4–6 every 2–3 weeks for oily, large-pore skin.

Selling it right

Best for oily skin, blackheads, dull tone, enlarged pores, and anyone with a function in 2–3 days. Not for very dry/sensitive skin, active inflamed acne or recent isotretinoin. Bundle it with a bridal or event package.

Hands-on today

- Perform a full carbon facial on a live client under supervision
- Practise even carbon application (the most common mistake is a thick, patchy layer)
- Write the treatment into the client record with settings

Safety & contraindications

- Never let carbon enter the eyes — cover and goggle
- Dry the carbon fully before firing

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Zero downtime, instant glow
- Best for oily/large pores
- 4–6 sessions, 2–3 weeks apart
- Perfect pre-event treatment

Homework

Design a bridal glow package: which treatments, in what order, over 8 weeks.

Grooming & professional tip

Event treatments create urgency by themselves. Keep a 'function loading' calendar and remind clients 3 weeks before festivals and wedding season.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 20

20 of 30 · Sundays holiday

Hydrafacial & medifacials

హైడ్రాఫేషియల్ & మెడిఫేషియల్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Run the complete Hydrafacial protocol with correct tips and serums
- Choose boosters by skin concern
- Maintain the machine correctly

THEORY · థియరీ

The three steps

Cleanse + peel (gentle glycolic/salicylic solution loosens dead cells), extract + hydrate (vortex vacuum lifts debris out of pores while delivering hydrating serum), fuse + protect (antioxidants, peptides, hyaluronic acid pushed in). Optional boosters and LED finish. Painless, no downtime, immediate plumping and glow.

Tips, serums and boosters

Different tips for exfoliation, extraction and infusion; suction levels adjusted for sensitive areas (nose higher, cheeks lower). Boosters: brightening for pigment, clarifying for acne, hydrating for dry, anti-ageing peptides for mature skin. Neck and décolleté should be included — clients notice the line otherwise.

Machine care

Prime the lines, check serum levels before the client arrives, dispose of single-use tips, flush and disinfect after every client, and log consumable usage. A badly maintained machine gives weak suction and a flat result — and clients feel the difference immediately.

Hands-on today

- Complete Hydrafacial on a live client, all three steps, 45 minutes
- Change tips and serums correctly; flush the system after
- Compare before/after pore photos on the analyser

Safety & contraindications

- Avoid over-suction on thin skin around the eyes and on rosacea
- Skip active inflamed acne areas; do not force extraction

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- 3 steps: cleanse-peel, extract-hydrate, fuse-protect
- Include neck & décolleté
- Single-use tips only
- Monthly maintenance sessions

Homework

Write a booster selection chart for 5 different client profiles.

Grooming & professional tip

Show clients the used-serum cup at the end. Seeing what came out of their pores sells the monthly package better than any brochure.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 21

21 of 30 · Sundays holiday

MNRF & fractional CO2 assist — texture & scars

MNRF & ఫ్రాక్షనల్ CO2 — టెక్స్చర్ & స్కార్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Understand microneedling RF and fractional CO2 and their differences
- Prepare, assist and manage the client through a resurfacing session
- Deliver the full post-procedure care plan

THEORY · థియరీ

MNRF

Insulated microneedles deliver radiofrequency heat at a set depth in the dermis, creating controlled thermal zones without damaging the epidermis — this makes it relatively safe in dark skin. Uses: acne scars, open pores, texture, mild laxity, stretch marks. 3–4 sessions, 4–6 weeks apart. Downtime 1–2 days of redness and grid marks.

Fractional CO2

Ablative laser vaporises microcolumns of skin, leaving untreated skin in between to heal fast. Strongest resurfacing option for scars and texture, but highest PIH risk in Fitzpatrick IV–V — conservative settings, priming, and strict post-care are essential. Downtime 5–7 days of redness, bronzing and peeling.

Your role

Numbing cream 45–60 minutes before with correct occlusion and timing, clean removal, set up smoke evacuator and cooling, assist during the pass, ice after, apply barrier ointment, and give the aftercare kit. Follow-up call at 24 h and 72 h is where complications are caught early.

Hands-on today

- Assist a full MNRF session and document depths and passes
- Apply and time numbing cream correctly on a volunteer
- Write the 7-day post-resurfacing care plan

Safety & contraindications

- Confirm no active infection, isotretinoin in 6 months, or keloid tendency
- Any sign of infection (pain, pus, spreading redness) → doctor the same day

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- MNRF: safer in dark skin, 3–4 sessions
- CO2: strongest, highest PIH risk
- Numbing 45–60 min
- 24 h and 72 h follow-up calls

Homework

Prepare the aftercare kit list and instruction card for a CO2 client.

Grooming & professional tip

The follow-up call is a clinical tool, not politeness. Catching one infection early protects a client, your reputation and the clinic.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 22

22 of 30 · Sundays holiday

Anti-ageing basics — Botox, fillers, threads, HIFU

యాంటీ-ఏజింగ్ బేసిక్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Explain each anti-ageing option accurately to a client
- Assist the doctor correctly in injectable procedures
- Know your boundary — what a therapist must never do

THEORY · థియరీ

What ageing looks like

Static and dynamic lines, volume loss (temples, cheeks, under-eye), skin laxity, jowls, texture and pigment change. Each needs a different tool: dynamic lines → botulinum toxin; volume → fillers; laxity → HIFU/RF/threads; texture/pigment → lasers and peels. A good plan combines two or three.

The procedures

Botulinum toxin: relaxes muscle, effect in 3–5 days, full at 2 weeks, lasts 4–6 months. Fillers: hyaluronic acid gel, instant volume, 9–18 months, reversible. Threads: PDO lifting + collagen, 12–18 months. HIFU: focused ultrasound to the SMAS, one session, lift builds over 2–3 months, lasts 12–18 months.

Your role and boundary

You prepare: numbing, cleansing, marking assistance, tray set-up (sterile), ice, documentation and aftercare instructions. You NEVER inject. Aftercare: no lying down 4 h, no rubbing, no gym or heat for 24–48 h, no facial for 2 weeks after fillers; report asymmetry, blanching or severe pain immediately — vascular occlusion is an emergency.

Hands-on today

- Set up a sterile injectable tray and be audited
- Observe a doctor-led session and write the full step log
- Practise the aftercare explanation for toxin and filler

Safety & contraindications

- Never inject anything — this is medical practice
- Blanching, severe pain or vision change after filler = emergency, doctor immediately

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Toxin: 3–5 days onset, 4–6 months
- Fillers: instant, reversible
- HIFU: one session, builds 2–3 months
- Therapists assist, never inject

Homework

Write a comparison card: toxin vs filler vs threads vs HIFU — result, onset, duration, downtime.

Grooming & professional tip

Clients ask therapists what they would never ask a doctor. Know these answers cold — you are the bridge that converts a consultation.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 23

23 of 30 · Sundays holiday

Mesotherapy, glutathione & skin boosters — assist

మెసోథెరపీ, గ్లూటాథియోన్ & స్కిన్ బూస్టర్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Understand mesotherapy and skin boosters and their realistic results
- Prepare and assist these sessions safely
- Counsel honestly about IV brightening

THEORY · థియరీ

Mesotherapy

Micro-injections of vitamins, amino acids, antioxidants and hyaluronic acid into the superficial dermis for hydration, glow and mild pigment improvement. 4–6 sessions, 2–3 weeks apart. Mild redness and tiny bumps for a few hours. Often combined with a facial or peel series.

Skin boosters

Injectable stabilised hyaluronic acid (not a volumiser) for deep hydration, fine-line softening and skin quality — face, neck, hands. 2–3 sessions a month apart, then 6-monthly maintenance. Good for brides and for dehydrated, dull mature skin.

Glutathione — the honest version

Glutathione is an antioxidant. Oral and topical use is cosmetic support. IV glutathione is a medical procedure with real risks and no guaranteed permanent whitening; it is never promised or administered by a therapist, and results fade without maintenance and sun protection. Counsel clients away from unrealistic 'permanent fairness' expectations — refer them to the doctor for a safe plan.

Hands-on today

- Prepare a sterile mesotherapy tray and assist one session
- Write realistic expectation scripts for glutathione enquiries
- Document a 6-session mesotherapy plan

Safety & contraindications

- Sterile technique at all times; single-use needles
- No IV procedures by therapists, ever
- Report any allergic reaction (swelling, itching, breathlessness) immediately

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Meso: 4–6 sessions, 2–3 weeks apart
- Boosters = hydration, not volume
- IV glutathione = medical, never promised
- Maintenance + sunscreen or results fade

Homework

Write the 'fairness expectation' conversation you will use, in Telugu, in under 8 lines.

Grooming & professional tip

Being the professional who says 'idi permanent kaadu' builds more long-term revenue than the one who oversells and loses the client in 3 months.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Complications, emergencies & how to handle them

కాంప్లికేషన్స్ & ఎమర్జెన్సీ మేనేజ్‌మెంట్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Recognise the common complications early
- Act correctly in the first 10 minutes
- Document and escalate professionally

THEORY · థియరీ

Common complications

Burns (peel/laser too deep or overlapping), blistering, PIH, hypopigmentation, persistent erythema, contact dermatitis, acne flare/purge, infection (bacterial or herpes reactivation), bruising and swelling after injectables, and the rare but critical vascular occlusion with fillers.

First response

Stop the procedure. Cool with saline-soaked gauze or a cold pack (never ice directly on a burn). Do not apply random creams. Call the doctor immediately. Take photographs. Keep the client calm — your tone decides whether this becomes a complaint or a managed event. Never argue or blame.

Prevention and documentation

90% of complications are prevented by: correct history, patch/test shots, conservative settings, single-pass discipline, correct timing and the client actually following aftercare. Document every session's settings and every complication in the record, with photos and the doctor's instructions. Follow up daily until resolved.

Hands-on today

- Simulate three scenarios: peel burn, laser blister, allergic reaction — act out the first 10 minutes
- Locate the emergency kit, doctor's number and escalation chart
- Write an incident report for a simulated complication

Safety & contraindications

- Never hide a complication — early escalation saves skin and reputation
- Anaphylaxis signs (breathlessness, facial swelling) = emergency, call for help immediately

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Stop → cool → call doctor → photograph
- No random creams on a burn
- Document everything
- Daily follow-up until resolved

Homework

Memorise the escalation chart and write it out from memory.

Grooming & professional tip

Clients remember how you behaved when something went wrong far longer than a perfect result. Calm, honest, present — that is the whole job.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 25

25 of 30 · Sundays holiday

Treatment planning, packages & case studies

ట్రీట్‌మెంట్ ప్లానింగ్, ప్యాకేజీలు & కేస్ స్టడీస్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Build a complete, staged treatment plan for a real client
- Combine treatments safely with correct gaps
- Present a package the client understands and accepts

THEORY · థియరీ

Planning framework

Assess → prioritise (what bothers the client most AND what must be treated first clinically) → prime → active phase → maintenance. Example acne + marks + scars: control acne (8 weeks) → treat marks (peels, 6 weeks) → treat scars (MNRF, 4 sessions) → maintenance facials + home care. Never run all three at once.

Safe combinations and gaps

Peel and LHR: 7 days apart. Peel and MNRF: 2 weeks. Facial and peel: same day only if the facial is gentle. Toxin and facial: 2 weeks. Never two resurfacing procedures in the same fortnight. Write the calendar down for the client.

Presenting a package

Show the plan on paper with dates and expected milestones. Explain what happens if they skip sessions. Give one recommended plan and one lighter alternative — two options convert better than five. Let the doctor approve any pricing.

Hands-on today

- Build 3 full plans from real client files (acne, melasma, bridal)
- Present one plan to the trainer as if to a client
- Draw the 8-week calendar for each plan

Safety & contraindications

- Every plan must be approved by the dermatologist before it is promised
- Re-assess at every session — plans change with the skin

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Prioritise clinically, not emotionally
- Respect treatment gaps
- Give 2 options, not 5
- Milestones keep clients in the plan

Homework

Write a complete 12-week bridal plan with dates, treatments and home care.

Grooming & professional tip

A written plan with dates is the single strongest retention tool in aesthetics. Clients follow calendars; they forget conversations.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 26

26 of 30 · Sundays holiday

Client communication, objections & retention

క్లయింట్ కమ్యూనికేషన్ & రిటెన్షన్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Handle the five most common objections confidently
- Communicate realistic expectations without losing the sale
- Build a retention system that runs on its own

THEORY · థియరీ

The five objections

'Price ekkuva' → break into sessions and compare with monthly spends; show the plan value. 'Time ledu' → offer evening/Saturday slots and shorter protocols. 'Pain untunda?' → explain numbing, cooling, and the honest sensation. 'Results permanent aa?' → be honest about maintenance. 'Alochisthanu' → agree, then book a provisional slot and send the plan on WhatsApp.

Expectation setting

Always state: number of sessions, time to first visible change, realistic percentage improvement, downtime, and what happens without home care. Write it down. A client who hears 'six sessions, first change in six weeks, 60–70%' is a happy client at session three. A client who heard 'don't worry' is a complaint.

Retention system

Rebook before they leave. Send aftercare on WhatsApp the same evening. Follow-up call at 48 hours. Reminder 3 days before the next session. Progress photos every 3 sessions. Birthday and festival messages. A 'we miss you' message at 60 days of no visit. All of this is automated in our WhatsApp agent — your job is to feed it accurate data.

Hands-on today

- Role-play all five objections, twice each, with feedback
- Write your own expectation-setting paragraph and memorise it
- Practise a 48-hour follow-up call script

Safety & contraindications

- Never counter an objection with a medical promise
- Never discount below the clinic's approved price to close a sale

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Rebook before they leave
- 48-hour follow-up call
- Progress photos every 3 sessions
- Write expectations down

Homework

Record yourself answering the five objections on your phone and listen back.

Grooming & professional tip

The most profitable sentence in a clinic is: 'Next session ki e roju convenient?' — asked before the client stands up.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 27

27 of 30 · Sundays holiday

Clinic operations, WhatsApp flow & aftercare system

క్లినిక్ ఆపరేషన్స్ & WhatsApp ఫ్లో

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Run the day: bookings, slots, room turnover, stock
- Work with the WhatsApp AI agent and the staff dashboard
- Close the day with clean records and follow-ups done

THEORY · థియరీ

The clinic day

Open: switch on and check machines, prime consumables, review the day's appointments and prepare files. During: keep to slot times, turn the room over in 5–7 minutes, log every treatment as it happens. Close: sterilise, restock, reconcile the day's records, list tomorrow's confirmations, report low stock.

WhatsApp agent and dashboard

Our AI assistant answers 24×7, books slots, sends reminders, aftercare and follow-ups, and creates a lead card with a 'call prep' note. Staff open dermaluxe.ai/staff.html, see hot leads first, call from the dashboard, set status (new → contacted → booked → visited) and write a call note after every conversation. If it is not in the dashboard, it did not happen.

Stock and machine care

Track consumables (tips, serums, peels, gloves, needles) weekly with minimum levels. Log machine service dates and calibration. A missing consumable at 6 pm costs a client, not just a sale.

Hands-on today

- Run a mock clinic day: 4 appointments, one walk-in, one complaint
- Use the staff dashboard: update statuses, add call notes for 5 leads
- Do a stock count and write the indent

Safety & contraindications

- Never double-book a laser slot — rushed laser work causes injuries
- Confirm identity and consent before every procedure

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Room turnover: 5–7 min
- Log as you go, not at night
- Dashboard note after every call
- Weekly stock count

Homework

Write your ideal clinic opening and closing checklists (10 points each).

Grooming & professional tip

Clinics do not lose money on treatments — they lose it on no-shows, missed follow-ups and empty evening slots. Fix those three and you are valuable anywhere.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 28

28 of 30 · Sundays holiday

Revision + full live-client day

రివిజన్ + లైవ్ క్లయింట్ డే

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Work a full day as the clinic therapist under observation
- Handle real clients start to finish without prompting
- Identify your own remaining weak areas

THEORY · థియరీ

How today works

You are rostered like a working therapist: your own clients, your own room, your own records. The trainer observes and intervenes only for safety. Every procedure must include consultation, consent check, analysis, procedure, aftercare explanation, record entry and rebooking.

What is being assessed

Hygiene and set-up, client handling and language, technical technique, parameter choice and justification, safety discipline, aftercare counselling, documentation, and time management. You will get written feedback on each.

Self-review

After each client, note one thing that went well and one to fix. This habit — quick, honest self-review — is what separates a 2-year therapist from a 10-year one.

Hands-on today

- Minimum 3 full client journeys today (facial/peel, LHR, device treatment)
- Complete records for each and submit for review
- Write your self-review after each client

Safety & contraindications

- Ask for the trainer before any parameter you are unsure about
- Never rush to finish a client before a break

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Full journey every time
- Records submitted same day
- One win + one fix per client

Homework

List your three weakest areas and the exact drill you will do for each before the exam.

Grooming & professional tip

Tomorrow's employer will not ask your marks. They will ask: can you run a room alone? Today is your rehearsal for that answer.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Mock practical, viva prep & portfolio

మాక్ ప్రాక్టికల్, వైవా ప్రిపరేషన్ & పోర్ట్‌ఫోలియో

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Complete a full mock practical under exam conditions
- Answer viva questions clearly and confidently
- Assemble a professional portfolio for job interviews

THEORY · థియరీ

Exam format

Practical (60%): set-up and hygiene, consultation and consent, one assigned procedure performed fully, aftercare and documentation. Viva (40%): skin anatomy, Fitzpatrick and safety, peel and laser parameters, complication management, and a case-plan question. Pass mark 70%.

Viva technique

Answer in structure: what it is → how it works → who it suits → contraindications → parameters → aftercare. If you don't know, say 'Idi naaku ఖచ్చితంగా teliyadu, doctor ni confirm chestanu' — honesty scores better than a wrong confident answer, and it is exactly what we want in the clinic.

Portfolio

Assemble: certificate, before/after photo sets (with consent), your treatment logs, the protocols you wrote, a one-page CV and a short self-introduction video. Employers in aesthetics hire on demonstrated hands, not degrees.

Hands-on today

- Full mock practical under exam conditions with scoring
- Rapid-fire viva: 25 questions with the trainer
- Build your portfolio folder and CV draft

Safety & contraindications

- Exam or not, safety failures are an automatic fail — goggles, consent, hygiene

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Practical 60% + viva 40%, pass 70%
- Structure every viva answer
- Honesty > guessing
- Portfolio = photos + logs + CV

Homework

Finish your CV and record a 60-second self-introduction in Telugu and English.

Grooming & professional tip

Bring your portfolio to the interview on your phone AND printed. The therapist who can show 10 before/afters is hired before the one who talks.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్

Day 30

30 of 30 · Sundays holiday

Final practical exam, viva & career guidance

ఫైనల్ ప్రాక్టికల్, వైవా & కెరీర్ గైడెన్స్

TODAY'S OBJECTIVES · ఈ రోజు లక్ష్యాలు

- Pass the final practical and viva
- Receive your certificate and portfolio feedback
- Plan your next 6 months in the aesthetics industry

THEORY · థియరీ

Today's schedule

Morning: practical exam (assigned procedure, full journey, scored live). Afternoon: viva. Evening: results, certificate presentation, group photo, and placement discussion for eligible trainees.

Career paths

Aesthetic therapist in a doctor-led clinic (our own clinics hire first from each batch), laser technician, clinic manager, product trainer with an aesthetic brand, or your own studio in partnership with a dermatologist. Salary grows with proven hands and retained clients, not with time served.

The next 6 months

Keep learning one new skill a month. Keep photographing your work. Keep a settings diary. Ask for the difficult cases. Stay connected to the academy group — we share new protocols, machine updates and job openings there. Refresher sessions are open to all alumni.

Hands-on today

- Final practical exam (scored)
- Final viva (scored)
- Certificate presentation and portfolio review

Safety & contraindications

- Carry the safety discipline you learned here into every clinic you work in — it is the profession's core

KEY POINTS TO REMEMBER · గుర్తుంచుకోవలసినవి

- Pass mark 70%
- Certificate + internship letter (2-month course)
- Alumni group for jobs and updates
- Priority hiring at our locations

Homework

Write your 6-month goal: role, skill, income and one measurable target.

Grooming & professional tip

You are now responsible for someone's skin and confidence. Stay humble, stay clean, keep asking questions — that is what makes a professional the clinic cannot replace.

SELF-CHECK · సెల్ఫ్ చెక్

- Did I follow hygiene and consent for every client today?
- Which step did I need help with — and what will I drill tonight?
- Did I write my records and settings before leaving?

Trainer sign: _____ Trainee sign: _____

MY NOTES · నా నోట్స్